

from outside the head. The voices may be familiar, and often make insulting statements. The voices may be single or multiple. Voices speaking directly to the individual or commenting on his or her ongoing behavior are particularly characteristic. Command hallucinations may be obeyed, at times creating danger for the individual or others. Occasionally, the auditory hallucinations are of sounds rather than voices. Tactile hallucinations may be present, and typically involve electrical, tingling, or burning sensations. Somatic hallucinations, such as the sensation of snakes crawling inside the abdomen, are occasionally experienced. Visual, gustatory, and olfactory hallucinations also occur, but with less frequency, and, in the absence of auditory hallucinations, always raise the possibility of an Organic Mental Disorder.

Other perceptual abnormalities include sensations of bodily change and hypersensitivity to sound, sight, and smell.

Affect. The disturbance often involves blunting, flattening, or inappropriateness of affect. In blunted affect there is severe reduction in the intensity of affective expression. In flat affect there are virtually no signs of affective expression; the voice is usually monotonous and the face, immobile. The individual may complain that he or she no longer responds with normal emotional intensity or, in extreme cases, no longer has feelings. In inappropriate affect, the affect is clearly discordant with the content of the individual's speech or ideation. For example, while discussing being tortured by electrical shocks, an individual with Schizophrenia, Paranoid Type, laughs or smiles. Sudden and unpredictable changes in affect involving inexplicable outbursts of anger may occur.

Although these affective disturbances are almost invariably present, their usefulness in making the diagnosis is limited because their presence is often difficult to judge except when present in extreme form. Furthermore, the antipsychotic drugs have effects that may appear similar to the affective blunting and flattening seen in Schizophrenia.

Sense of self. The sense of self that gives the normal person a feeling of individuality, uniqueness, and self-direction is frequently disturbed. This is sometimes referred to as a loss of ego boundaries and is frequently manifested by extreme perplexity about one's own identity and the meaning of existence, or by some of the specific delusions described above, particularly those involving control by an outside force.

Volition. Nearly always there is some disturbance in self-initiated, goal-directed activity, which may grossly impair work or other role functioning. This may take the form of inadequate interest or drive or inability to follow a course of action to its logical conclusion. Pronounced ambivalence regarding alternative courses of action can lead to near cessation of goal-directed activity.

Relationship to the external world. Frequently there is a tendency to withdraw from involvement with the external world and to become preoccupied with egocentric and illogical ideas and fantasies in which objective facts are obscured,

distorted, or excluded. When severe, this condition has been referred to as "autism." Family members or friends frequently comment that the individual seems preoccupied, in his or her own world, and emotionally detached from others.

Psychomotor behavior. Various disturbances in psychomotor behavior are observed, particularly in the chronically severe and acutely florid forms of the disorder. A marked decrease may occur in reactivity to the environment, with a reduction in spontaneous movements and activity. In extreme cases the individual appears unaware of the nature of the environment (as in catatonic stupor); may maintain a rigid posture resisting efforts to be moved (as in catatonic rigidity); may make apparently purposeless and stereotyped, excited motor movements not influenced by external stimuli (as in catatonic excitement); may voluntarily assume inappropriate or bizarre postures (as in catatonic posturing); or resist and actively counteract instructions or attempts to be moved (as in catatonic negativism). In addition, mannerisms, grimacing, or waxy flexibility may be present.

Associated features. Almost any symptom can occur as an associated feature. The individual may appear perplexed, disheveled, or eccentrically groomed or dressed. Abnormalities of psychomotor activity—e.g., pacing, rocking, or apathetic immobility—are common. Frequently, there is poverty of speech, that is, a restriction in the amount of spontaneous speech, so that replies to questions tend to be brief, concrete, and unelaborated. Ritualistic or stereotyped behavior associated with magical thinking often occurs. Dysphoric mood is common, and may take the form of depression, anxiety, anger, or a mixture of these. Depersonalization, derealization, ideas of reference, and illusions are often present, as are hypochondriacal concerns that may or may not be delusional. Typically, no disturbance in sensorium is evident, although during a period of exacerbation the individual may be confused and even disoriented, or have memory impairment.

Age at onset. Onset is usually during adolescence or early adulthood.

Course. As noted previously, the diagnosis of Schizophrenia requires that continuous signs of the illness have lasted for at least six months which always includes an active phase of psychotic symptoms, and may or may not include prodromal or residual phases.

The development of the active phase of the illness is usually preceded by a *prodromal phase* in which there is a clear deterioration in a previous level of functioning. This phase is characterized by social withdrawal, impairment in role functioning, peculiar behavior, impairment in personal hygiene and grooming, blunted or inappropriate affect, disturbances in communication, bizarre ideation, and unusual perceptual experiences. A change in personality is often noted by friends or relatives. The length of this prodromal phase is extremely variable, and its onset may be difficult to date accurately. The prognosis is especially poor when the prodromal phase has taken an insidious, downhill course over many years.

During the *active phase* psychotic symptoms—e.g., delusions, hallucinations, loosening of associations, incoherence, poverty of content of speech, markedly illogical thinking, and behavior that is grossly disorganized or catatonic—are prominent. The specific psychotic symptoms are noted in criterion A of the diagnostic criteria (p. 188). The onset of the active phase, either initially or as an exacerbation of a preexisting active phase, is often associated with a psychosocial stressor.

Usually a *residual phase* follows the active phase of the illness. The clinical picture of this phase is similar to that of the prodromal phase, although affective blunting or flattening and impairment in role functioning tend to be more common in the residual phase. During the residual phase some of the psychotic symptoms, such as delusions or hallucinations, may persist, but are no longer accompanied by strong affect.

A complete return to premorbid functioning is unusual—so rare, in fact, that some clinicians would question the diagnosis. However, there is always the *possibility* of full remission or recovery, although its frequency is unknown. The most common course is one of acute exacerbations with increasing residual impairment between episodes.

Numerous studies have indicated a group of factors associated with good prognosis: absence of premorbid personality disturbance, adequate premorbid social functioning, precipitating events, abrupt onset, onset in mid-life, a clinical picture that involves confusion, and a family history of Affective Disorder.

Because a knowledge of course is very important for planning treatment, and because differences in course may reflect fundamental differences among Schizophrenic Disorders, a separate digit is provided for coding the course as Subchronic, Chronic, Subchronic with Acute Exacerbation, Chronic with Acute Exacerbation, or In Remission (for criteria, see p. 192).

Since a six-month duration of illness is required for the diagnosis, there is no acute subtype. (The DSM-III diagnosis of Schizophreniform Disorder is the nearest equivalent to the DSM-II and ICD-9-CM concepts of Acute Schizophrenic Episode. Frequently, an episode of Schizophreniform Disorder will persist for more than six months, in which case the diagnosis should be changed to Schizophrenia.)

Impairment. Invariably there is impairment in several areas of routine daily functioning, such as work, social relations, and self-care. Supervision may be required to ensure that nutritional and hygienic needs are met and to protect the individual from the consequences of poor judgment, cognitive impairment, or actions based on delusions or in response to hallucinations. Between episodes of illness there may be no disability or the extent of disability ranges from none to disability so severe that institutional care is required.

Complications. Although violent acts performed by individuals with this disorder often attract public attention, whether the frequency of such acts is greater than in the general population is not known. The life expectancy of individuals with Schizophrenia is shorter than that of the general population because of an increased suicide rate and death from a variety of other causes—

some, at least in the past, associated with institutional care. Other causes of death are associated with the economically deprived environments in which many individuals with this disorder live.

Premorbid personality. The premorbid personalities of individuals who develop Schizophrenia are often described as suspicious, introverted, withdrawn, or eccentric. Such individuals may meet the criteria for Paranoid, Schizoid, Schizotypal, or Borderline Personality Disorder. In such cases, since it can have prognostic significance, the Personality Disorder should be noted on Axis II, followed by the phrase "(Premorbid)."

Predisposing factors. The diagnosis is made more commonly among the lower socioeconomic groups. The reasons for this are unclear, but may involve downward social drift, lack of upward socioeconomic mobility, and high stress.

Certain patterns of family interaction have been hypothesized as predisposing to the development, onset, relapse or chronicity of Schizophrenia. The interpretation of the evidence supporting these hypotheses is controversial.

Prevalence. Studies in Europe and Asia, using a relatively narrow concept of Schizophrenia, have found a lifetime prevalence rate of from 0.2% to almost 1%. Studies done in the United States that have used broader criteria and surveyed urban populations have reported higher rates.

Sex ratio. The disorder is apparently equally common in males and in females.

Familial pattern. All investigators have found a higher prevalence of the disorder among family members. This includes studies in which the adopted offspring of individuals with Schizophrenia have been reared by parents who do not have Schizophrenia. Twin studies consistently show a higher concordance rate of Schizophrenia for monozygotic than dizygotic twins, and dizygotic twins have the same concordance rate as siblings who are not twins. However, being a monozygotic twin does not in itself predispose to the development of Schizophrenia. Although genetic factors have been proven to be involved in the development of the illness, the existence of a substantial discordance rate, even in monozygotic twins, indicates the importance of nongenetic factors.

Differential diagnosis. **Organic Mental Disorders** often present with symptoms that suggest Schizophrenia, such as delusions, hallucinations, incoherence, and blunted or inappropriate affect. In particular, **Organic Delusional Syndromes, such as those due to amphetamines or phencyclidine**, may cross-sectionally be identical in symptomatology with Schizophrenia. Even though an active phase of Schizophrenia may begin with confusion, the presence of disorientation or memory impairment strongly suggests an Organic Mental Disorder. (Of course, it is possible for an individual with Schizophrenia to have a superimposed Organic Mental Disorder.)

Paranoid Disorders are distinguished from Schizophrenia by the absence of prominent hallucinations, incoherence, loosening of associations, or bizarre delusions, such as delusions of being controlled or thought broadcasting.

In **Affective Disorders** there often are withdrawal and deterioration in functioning. These should not be mistaken for prodromal signs of Schizophrenia. In Affective Disorders the development of delusions or hallucinations follows a period of affective disturbance. For this reason the diagnosis of Schizophrenia is not made unless an affective syndrome, if present, developed *after* any psychotic symptoms or was brief in duration relative to the duration of the characteristic psychotic symptoms. The differential diagnosis of Schizophrenia from the psychotic forms of the Affective Disorders, particularly Bipolar Disorder, is of special importance because of the different long-term treatment implications. A manic episode with anger and paranoid delusions needs to be distinguished from Schizophrenia, Paranoid Type. However, an **Atypical Affective Disorder** or **Adjustment Disorder with Depressed Mood** may be superimposed on Schizophrenia, Residual Type. An example would be an individual with Schizophrenia, Residual Type, Chronic, who could develop a major depressive episode of several months' duration without any psychotic symptoms. In such a case both Schizophrenia and Atypical Depression should be diagnosed.

The diagnosis **Schizoaffective Disorder** should be made whenever the clinician is unable to make a differential diagnosis between Schizophrenia and Affective Disorder. Although no criteria for Schizoaffective Disorder are provided in this manual, several examples of clinical situations in which this diagnosis might be appropriate are given on p. 202.

In **Schizophreniform Disorder**, by definition the duration of the illness is less than six months. The cross-sectional symptom picture may be indistinguishable from Schizophrenia, but emotional turmoil and confusion are more likely to occur in Schizophreniform Disorder. It should be noted that the six-month duration of illness required for Schizophrenia refers to a continuous period of illness. Thus, an individual with several episodes of Schizophreniform Disorder from each of which there has been full recovery would not be diagnosed as having Schizophrenia merely because the total period of illness exceeded six months.

Atypical Psychosis is diagnosed when there is a nonaffective psychotic disorder but there is insufficient information to make a diagnosis of Schizophrenia. It is also diagnosed in those unusual instances in which one of the psychotic symptoms of Schizophrenia such as an encapsulated delusion of bodily change is present, but there is apparently no deterioration from a previous level of functioning.

In a **Pervasive Developmental Disorder**, the cross-sectional picture, particularly of the Residual State, may resemble Schizophrenia, Residual Type. However, there is no history of delusions, hallucinations, or incoherence.

In **Obsessive Compulsive Disorder**, **Hypochondriasis**, and more rarely **Phobic Disorder**, in order to account for the symptoms the individual may develop overvalued ideas that are difficult to distinguish from delusions. However, individuals with these disorders recognize, at least to some degree, that their symptoms and thinking are irrational, even if they are dominated by them.

In **Factitious Disorder with Psychological Symptoms**, “psychotic” symptoms are under the individual’s voluntary control and are likely to be present only when the individual thinks he or she is being observed.

In **Personality Disorders, especially Schizotypal, Borderline, Schizoid, and Paranoid** types transient psychotic symptoms may occur. However, a return within hours or days to the usual level of functioning distinguishes these disorders from Schizophrenia. It is more difficult to distinguish severe forms of Paranoid and Schizotypal Personality Disorders from Schizophrenia because of the difficulty in determining whether the paranoid ideation is of delusional intensity and whether the oddities of communication and perception are severe enough to meet the criteria for Schizophrenia. Furthermore, it is often difficult to differentiate the prodromal phase of Schizophrenia from the manifestations of some of the Personality Disorders since both Personality Disorders and Schizophrenia usually develop during adolescence or early adult life.

Beliefs or experiences of members of religious or other subcultural groups may be difficult to distinguish from delusions or hallucinations. When such experiences are shared and accepted by a subcultural group they should not be considered evidence of psychosis.

In **Mental Retardation**, low level of social functioning, oddities of behavior, and impoverished affect and cognition all may suggest Schizophrenia. Both diagnoses should be made in the same individual only when there is certainty that the symptoms suggesting Schizophrenia, such as delusions or hallucinations, are definitely present and are not the result of difficulties in communication.

Diagnostic criteria for a Schizophrenic Disorder

A. At least one of the following during a phase of the illness:

- (1) bizarre delusions (content is patently absurd and has no possible basis in fact), such as delusions of being controlled, thought broadcasting, thought insertion, or thought withdrawal
- (2) somatic, grandiose, religious, nihilistic, or other delusions without persecutory or jealous content
- (3) delusions with persecutory or jealous content if accompanied by hallucinations of any type
- (4) auditory hallucinations in which either a voice keeps up a running commentary on the individual’s behavior or thoughts, or two or more voices converse with each other
- (5) auditory hallucinations on several occasions with content of more than one or two words, having no apparent relation to depression or elation
- (6) incoherence, marked loosening of associations, markedly illogical thinking, or marked poverty of content of speech if associated with at least one of the following:

- (a) blunted, flat, or inappropriate affect
- (b) delusions or hallucinations
- (c) catatonic or other grossly disorganized behavior

B. Deterioration from a previous level of functioning in such areas as work, social relations, and self-care.

C. Duration: Continuous signs of the illness for at least six months at some time during the person's life, with some signs of the illness at present. The six-month period must include an active phase during which there were symptoms from A, with or without a prodromal or residual phase, as defined below.

Prodromal phase: A clear deterioration in functioning before the active phase of the illness not due to a disturbance in mood or to a Substance Use Disorder and involving at least two of the symptoms noted below.

Residual phase: Persistence, following the active phase of the illness, of at least two of the symptoms noted below, not due to a disturbance in mood or to a Substance Use Disorder.

Prodromal or Residual Symptoms

- (1) social isolation or withdrawal
- (2) marked impairment in role functioning as wage-earner, student, or homemaker
- (3) markedly peculiar behavior (e.g., collecting garbage, talking to self in public, or hoarding food)
- (4) marked impairment in personal hygiene and grooming
- (5) blunted, flat, or inappropriate affect
- (6) digressive, vague, overelaborate, circumstantial, or metaphorical speech
- (7) odd or bizarre ideation, or magical thinking, e.g., superstitiousness, clairvoyance, telepathy, "sixth sense," "others can feel my feelings," overvalued ideas, ideas of reference
- (8) unusual perceptual experiences, e.g., recurrent illusions, sensing the presence of a force or person not actually present

Examples: Six months of prodromal symptoms with one week of symptoms from A; no prodromal symptoms with six months of symptoms from A; no prodromal symptoms with two weeks of symptoms from A and six months of residual symptoms; six months of symptoms from A, apparently followed by several years of complete remission, with one week of symptoms in A in current episode.

D. The full depressive or manic syndrome (criteria A and B of major depressive or manic episode), if present, developed after any psychotic symptoms, or was brief in duration relative to the duration of the psychotic symptoms in A.

E. Onset of prodromal or active phase of the illness before age 45.

F. Not due to any Organic Mental Disorder or Mental Retardation.

TYPES

The types reflect cross-sectional clinical syndromes. Some are less stable over time than others, and their prognostic and treatment implications are variable. The diagnosis of a particular type should be based on the predominant clinical picture that occasioned the evaluation or admission to clinical care.

295.1x Disorganized Type

The essential features are marked incoherence and flat, incongruous, or silly affect. There are no systematized delusions although fragmentary delusions or hallucinations in which the content is not organized into a coherent theme are common. Associated features include grimaces, mannerisms, hypochondriacal complaints, extreme social withdrawal, and other oddities of behavior.

This clinical picture is usually associated with extreme social impairment, poor premorbid personality, an early and insidious onset, and a chronic course without significant remissions.

In other classifications this type is termed Hebephrenic.

Diagnostic criteria for Disorganized Type

A type of Schizophrenia in which there are:

A. Frequent incoherence.

B. Absence of systematized delusions.

C. Blunted, inappropriate, or silly affect.

295.2x Catatonic Type

The essential feature is marked psychomotor disturbance, which may involve stupor, negativism, rigidity, excitement, or posturing. Sometimes there is rapid alternation between the extremes of excitement and stupor. Associated features include stereotypies, mannerisms, and waxy flexibility. Mutism is particularly common.

During catatonic stupor or excitement the individual needs careful supervision to avoid hurting self or others, and medical care may be needed because

of malnutrition, exhaustion, hyperpyrexia, or self-inflicted injury.

Although this type was very common several decades ago, it is now rare in Europe and North America.

Diagnostic criteria for Catatonic Type

A type of Schizophrenia dominated by any of the following:

- (1) catatonic stupor (marked decrease in reactivity to environment and/or reduction of spontaneous movements and activity) or mutism
- (2) catatonic negativism (an apparently motiveless resistance to all instructions or attempts to be moved)
- (3) catatonic rigidity (maintenance of a rigid posture against efforts to be moved)
- (4) catatonic excitement (excited motor activity, apparently purposeless and not influenced by external stimuli)
- (5) catatonic posturing (voluntary assumption of inappropriate or bizarre posture)

295.3x Paranoid Type

The essential features are prominent persecutory or grandiose delusions, or hallucinations with a persecutory or grandiose content. In addition, delusional jealousy may be present.

Associated features include unfocused anxiety, anger, argumentativeness, and violence. In addition, there may be doubts about gender identity or fear of being thought of as a homosexual, or being approached by homosexuals.

The impairment in functioning may be minimal if the delusional material is not acted upon, since gross disorganization of behavior is relatively rare. Similarly, affective responsiveness may be preserved. Often a stilted, formal quality, or extreme intensity in interpersonal interactions is noted.

The onset tends to be later in life than the other subtypes, and the features are more stable over time. If a biologically related family member of an individual who has this subtype also has Schizophrenia, there is some evidence that the subtype of the relative will also be paranoid.

Diagnostic criteria for Paranoid Type

A type of Schizophrenia dominated by one or more of the following:

- (1) persecutory delusions
- (2) grandiose delusions
- (3) delusional jealousy
- (4) hallucinations with persecutory or grandiose content

295.9x Undifferentiated Type

The essential features are prominent psychotic symptoms that cannot be classi-

fied in any category previously listed or that meet the criteria for more than one.

Diagnostic criteria for Undifferentiated Type

A. A type of Schizophrenia in which there are: Prominent delusions, hallucinations, incoherence, or grossly disorganized behavior.

B. Does not meet the criteria for any of the previously listed types or meets the criteria for more than one.

295.6x Residual Type

This category should be used when there has been at least one episode of Schizophrenia but the clinical picture that occasioned the evaluation or admission to clinical care is without prominent psychotic symptoms, though signs of the illness persist. Emotional blunting, social withdrawal, eccentric behavior, illogical thinking and loosening of associations are common. If delusions or hallucinations are present, they are not prominent and are not accompanied by strong affect.

The course of this type is either chronic or subchronic, since "acute exacerbation" by definition, involves prominent psychotic symptoms, and "in remission" implies no signs of the illness.

Diagnostic criteria for Residual Type

A. A history of at least one previous episode of Schizophrenia with prominent psychotic symptoms.

B. A clinical picture without any prominent psychotic symptoms that occasioned evaluation or admission to clinical care.

C. Continuing evidence of the illness, such as blunted or inappropriate affect, social withdrawal, eccentric behavior, illogical thinking, or loosening of associations.

Classification of course. The course of the illness is coded in the fifth digit:

(1) *Subchronic*. The time from the beginning of the illness, during which the individual began to show signs of the illness (including prodromal, active, and residual phases) more or less continuously, is less than two years but at least six months.

(2) *Chronic*. Same as above, but greater than two years.

(3) *Subchronic with Acute Exacerbation*. Reemergence of prominent psychotic symptoms in an individual with a subchronic course who has been in the residual phase of the illness.

(4) *Chronic with Acute Exacerbation*. Reemergence of prominent psychotic symptoms in an individual with a chronic course who has been in the residual phase of the illness.

(5) *In Remission*. This should be used when an individual with a history of Schizophrenia, now is free of all signs of the illness (whether or not on medication). The differentiation of Schizophrenia In Remission from no mental disorder requires consideration of the period of time since the last period of disturbance, the total duration of the disturbance, and the need for continued evaluation or prophylactic treatment.

When the course is noted as "in remission," the phenomenologic type should describe the last episode of Schizophrenia, e.g., 295.25 Schizophrenia, Catatonic Type, In Remission. When the phenomenology of the last episode is unknown, it should be noted as Undifferentiated.

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Paranoid Disorders

The essential features are persistent persecutory delusions or delusional jealousy, not due to any other mental disorder, such as a Schizophrenic, Schizophreniform, Affective, or Organic Mental Disorder. The Paranoid Disorders include Paranoia, Shared Paranoid Disorder, and Acute Paranoid Disorder.

The boundaries of this group of disorders and their differentiation from such other disorders as severe Paranoid Personality Disorder and Schizophrenia, Paranoid Type, are unclear.

The persecutory delusions may be simple or elaborate and usually involve a single theme or series of connected themes, such as being conspired against, cheated, spied upon, followed, poisoned or drugged, maliciously maligned, harassed, or obstructed in the pursuit of long-term goals. Small slights may be exaggerated and become the focus of a delusional system.

There may be only delusional jealousy ("conjugal paranoia"), in which an individual may become convinced without due cause, that his or her mate is unfaithful. Small bits of "evidence," such as disarrayed clothing or spots on the sheets, may be collected and used to justify the delusion.

Associated features. Common associated features include resentment and anger, which may lead to violence. Grandiosity and ideas or delusions of reference are common. Often there is social isolation, seclusiveness, or eccentricities of behavior. Suspiciousness, either generalized or focused on certain individuals, is common. Letter writing, complaining about various injustices, and instigation of legal action are frequent. These individuals rarely seek treatment, and often are brought for care by associates, relatives, or governmental agencies as a result of the individuals' angry or litigious activities.

Age at onset. Generally middle or late adult life.

Course. The course of Paranoia and Shared Paranoid Disorder is chronic with few, if any, exacerbations or periods of remission. The course of Acute Paranoid Disorder, by definition, is limited to six months' duration.

Impairment. Impairment in daily functioning is rare. Intellectual and occupational functioning are usually preserved, even when the disorder is chronic. Social and marital functioning, on the other hand, are often severely impaired.

Complications. None.

Predisposing factors. Immigration, emigration, deafness, and other severe

stresses may predispose to the development of a Paranoid Disorder. Individuals with Paranoid or Schizoid Personality Disorders may also have a greater likelihood of developing a Paranoid Disorder.

Prevalence. Paranoid Disorders are thought to be rare. However, Paranoia involving delusional jealousy may be more common.

Sex ratio and familial pattern. No information.

Differential diagnosis. In **Organic Delusional Syndromes**, particularly those induced by amphetamines, persecutory delusions are common.

In **Schizophrenia, Paranoid Type**, or **Schizophreniform Disorder**, there are certain symptoms, such as incoherence, marked loosening of associations, prominent hallucinations, and bizarre delusions (e.g., delusions of control, thought broadcasting, withdrawal, or insertion), that are not present in Paranoid Disorders. Although delusions that others are attempting to control the individual's behavior are common in both Paranoid and Schizophrenic Disorders, the experience of being controlled by alien forces suggests Schizophrenia or Schizophreniform Disorder. In addition, delusions in Schizophrenia are more likely to be fragmented and multiple rather than systematized, as in Paranoid Disorders.

In **Paranoid Personality Disorder** there may be paranoid ideation or pathological jealousy, but there are no delusions. Whenever an individual with a Paranoid Disorder has a preexisting Personality Disorder, including Paranoid Personality Disorder, the Personality Disorder should be listed on Axis II, followed by the phrase "Premorbid" in parentheses.

Diagnostic criteria for Paranoid Disorder

A. Persistent persecutory delusions or delusional jealousy.

B. Emotion and behavior appropriate to the content of the delusional system.

C. Duration of illness of at least one week.

D. None of the symptoms of criterion A of Schizophrenia (p. 188), such as bizarre delusions, incoherence, or marked loosening of associations.

E. No prominent hallucinations.

F. The full depressive or manic syndrome (criteria A and B of major depressive or manic episode, p. 213, and p. 208) is either not present, developed after any psychotic symptoms, or was brief in duration relative to the duration of the psychotic symptoms.

G. Not due to an Organic Mental Disorder.

297.10 Paranoia

The essential feature is the insidious development of a Paranoid Disorder with a permanent and unshakable delusional system accompanied by preservation of clear and orderly thinking. Frequently the individual considers himself or herself endowed with unique and superior abilities. Chronic forms of "conjugal paranoia" and Involutional Paranoid State should be classified here.

Diagnostic criteria for Paranoia

A. Meets the criteria for Paranoid Disorder (p. 196).

B. A chronic and stable persecutory delusional system of at least six months' duration.

C. Does not meet the criteria for Shared Paranoid Disorder.

297.30 Shared Paranoid Disorder

The essential feature is a persecutory delusional system that develops as a result of a close relationship with another person who already has a disorder with persecutory delusions. The delusions are at least partly shared. Usually, if the relationship with the other person is interrupted, the delusional beliefs will diminish or disappear. In the past this disorder has been termed *Folie à deux*, although in rare cases, more than two persons may be involved.

Diagnostic criteria for Shared Paranoid Disorder

A. Meets the criteria for Paranoid Disorder (p. 196).

B. Delusional system develops as a result of a close relationship with another person or persons who have an established disorder with persecutory delusions.

298.30 Acute Paranoid Disorder

The essential feature is a Paranoid Disorder of less than six months' duration. It is most commonly seen in individuals who have experienced drastic changes in their environment, such as immigrants, refugees, prisoners of war, inductees into military services, or people leaving home for the first time. The onset is usually relatively sudden and the condition rarely becomes chronic.

Diagnostic criteria for Acute Paranoid Disorder

A. Meets the criteria for Paranoid Disorder (p. 196).

B. Duration of less than six months.

C. Does not meet the criteria for Shared Paranoid Disorder (p. 197).

297.90 Atypical Paranoid Disorder

This is a residual category for Paranoid Disorders not classifiable above.

Psychotic Disorders Not Elsewhere Classified

This diagnostic class is for psychotic disorders that cannot be classified as either an Organic Mental Disorder or a Schizophrenic, Paranoid, or Affective Disorder. There are three specific categories: Schizophreniform Disorder, Brief Reactive Psychosis, and Schizoaffective Disorder. Finally, there is a residual category, Atypical Psychosis, for psychotic disorders that do not meet the criteria for any specific psychotic disorder. (For a definition of "psychotic," see p. 353.)

295.40 Schizophreniform Disorder

The essential features are identical with those of Schizophrenia with the exception that the duration, including prodromal, active and residual phases, is less than six months but more than two weeks. Schizophreniform Disorder is classified outside the category of Schizophrenic Disorders because evidence suggests a greater likelihood of emotional turmoil and confusion, a tendency toward acute onset and resolution, more likely recovery to premorbid levels of functioning, and the absence of an increase in the prevalence of Schizophrenia among family members compared with the general population. The six-month criterion has been chosen because several studies indicate that this is the best single way of differentiating these two disorders to maximize the difference in their external correlates. (In the past the term "Schizophreniform" has included cases that would be classified as Schizoaffective Disorder in this manual. Therefore, some of the conclusions drawn from that body of research may not apply to Schizophreniform Disorder as defined here.)

Differential diagnosis. Since the diagnostic criteria for **Schizophrenia** and Schizophreniform Disorder differ only in duration of illness, most of the discussion of differential diagnosis in the text for Schizophrenia (p. 186) applies equally to Schizophreniform Disorder, with the exception that the clinical picture in Schizophreniform Disorder is more often characterized by emotional turmoil, fear, confusion, and particularly vivid hallucinations.

Brief Reactive Psychosis differs from Schizophreniform Disorder in that the duration of the disturbance is less than two weeks (although secondary effects may persist longer). In addition, Brief Reactive Psychosis always follows a psychosocial stressor, which frequently is not present before the onset of Schizophreniform Disorder. If what appears to be a Brief Reactive Psychosis persists beyond two weeks, a diagnosis of Schizophreniform Disorder should be considered.

Atypical Psychosis should be diagnosed if the symptom picture is consistent with that of Schizophreniform Disorder but the duration is less than two weeks and the disturbance does not follow a psychosocial stressor.

Diagnostic criteria for Schizophreniform Disorder

A. Meets all of the criteria for Schizophrenia (p. 188) except for duration.

B. The illness (including prodromal, active, and residual phases) lasts more than two weeks but less than six months.

298.80 Brief Reactive Psychosis

The essential feature is the sudden onset of a psychotic disorder of at least a few hours' but no more than two weeks' duration, with eventual return to pre-morbid level of functioning. The psychotic symptoms appear immediately following a recognizable psychosocial stressor that would evoke significant symptoms of distress in almost anyone. The precipitating event may be any major stress, such as the loss of a loved one or the psychological trauma of combat. Invariably there is emotional turmoil, manifested by rapid shifts from one dysphoric affect to another without the persistence of any one affect.

In order not to misdiagnose this condition when the disturbance is actually due to a more pervasive disorder, such as Schizophrenia, this diagnosis should not be made if there was a period of increasing psychopathology immediately before the psychosocial stressor.

Associated features. Frequently perplexity and a feeling of confusion are present, which the individual may acknowledge or which can be judged from the way he or she responds to questions and requests.

Behavior may be bizarre and include peculiar postures, outlandish dress, screaming, or muteness. Suicidal or aggressive behavior may also be present. Speech may include inarticulate gibberish or repetition of nonsensical phrases. Affect is often inappropriate and volatile. Transient hallucinations or delusions are common. Silly or obviously confabulated answers may be given to factual questions. Disorientation and impairment in recent memory often occur.

Age at onset. The disorder usually appears in adolescence and early adulthood.

Course. Usually the psychotic symptoms clear in a day or two. By definition this diagnosis is not applicable if the psychotic symptoms persist for more than two weeks. Transient secondary effects, such as loss of self-esteem and mild depression, may persist beyond the two weeks, but there is eventually a full return to the premorbid level of functioning.

Impairment. Supervision may be required to ensure that nutritional and hygienic needs are met and that the individual is protected from the consequences of poor judgment, cognitive impairment, or acting on the basis of delusions.

Complications. None.

Predisposing factors. Preexisting psychopathology may predispose to the development of this disorder. Individuals with Paranoid, Histrionic, Narcissistic, Schizotypal, or Borderline Personality Disorders are thought to be particularly vulnerable to its development. By definition situations involving major stress predispose to development of this disorder.

Prevalence, sex ratio, and familial pattern. No information.

Differential diagnosis. Although by definition this diagnosis is not made if the psychotic symptoms persist for more than two weeks, the diagnosis can be made soon after the onset of the disturbance without waiting for the expected recovery. If the psychotic symptoms last more than two weeks, the diagnosis should be changed to either **Schizophreniform Disorder**, **Paranoid Disorder**, **Affective Disorder**, or **Atypical Psychosis**.

Organic Mental Disorders, particularly those involving **Delirium**, **Organic Delusional Syndrome**, or **Intoxication**, can be distinguished from this disorder only on the basis of historical or laboratory information that indicates a known organic factor.

Schizophreniform Disorder by definition requires a duration of more than two weeks, and frequently there is no precipitating psychosocial stressor.

Manic and major depressive episodes may follow a major psychosocial stressor. The diagnosis of a manic or major depressive episode preempts the diagnosis of Brief Reactive Psychosis, and should be made when the criteria for it are met whether or not it is associated with a psychosocial stressor.

Individuals with a **Personality Disorder** may, under stress, develop Brief Reactive Psychosis, in which case both diagnoses should be made.

An episode of **Factitious Disorder with Psychological Symptoms** may have apparently psychotic symptoms and may also be precipitated by a psychosocial stressor, but in such cases there is evidence that the symptoms are under voluntary control.

When **Malingering** presents with apparently psychotic symptoms, there is usually evidence that the illness was feigned for an understandable goal.

Diagnostic criteria for Brief Reactive Psychosis

A. Psychotic symptoms appear immediately following a recognizable psychosocial stressor that would evoke significant symptoms of distress in almost anyone.

B. The clinical picture involves emotional turmoil and at least one of the following psychotic symptoms:

- (1) incoherence or loosening of associations
- (2) delusions
- (3) hallucinations
- (4) behavior that is grossly disorganized or catatonic

C. The psychotic symptoms last more than a few hours but less than two weeks, and there is an eventual return to the premorbid level of functioning. (Note: The diagnosis can be made soon after the onset of the psychotic symptoms without waiting for the expected recovery. If the psychotic symptoms last more than two weeks, the diagnosis should be changed.)

D. No period of increasing psychopathology immediately preceded the psychosocial stressor.

E. The disturbance is not due to any other mental disorder, such as an Organic Mental Disorder, manic episode, or Factitious Disorder with Psychological Symptoms.

295.70 Schizoaffective Disorder

The term Schizoaffective Disorder has been used in many different ways since it was first introduced, and at the present time there is no consensus on how this category should be defined. Some of the cases that in the past were diagnosed as Schizoaffective Disorder would in this manual be diagnosed as Schizophreniform Disorder, Major Depression or Bipolar Disorder with Mood-congruent or Mood-incongruent Psychotic Features, or Schizophrenia with a superimposed Atypical Affective Disorder. Future research is needed to determine whether there is a need for this category, and if so, how it should be defined and what its relationship is to Schizophrenia and Affective Disorder.

The category is retained in this manual without diagnostic criteria for those instances in which the clinician is unable to make a differential diagnosis with any degree of certainty between Affective Disorder and either Schizophreniform Disorder or Schizophrenia. Before using the Schizoaffective Disorder category, the clinician should consider all of the diagnoses noted in the first paragraph above, particularly Major Affective Disorders with Psychotic Features.

Examples of cases that may appropriately be diagnosed as Schizoaffective Disorder include:

An episode of affective illness in which preoccupation with a mood-incongruent delusion or hallucination dominates the clinical picture when affective symptoms are no longer present.

An episode of illness in which currently there is a full affective syndrome with prominent mood-incongruent psychotic features but in which inadequate information about the presence of previous non-affective psychotic features makes it difficult to differentiate between Schizophrenia or Schizophreniform Disorder (with a superimposed Atypical Affective Disorder) and Affective Disorder.

298.90 Atypical Psychosis

This is a residual category for cases in which there are psychotic symptoms (delusions, hallucinations, incoherence, loosening of associations, markedly illog-

ical thinking, or behavior that is grossly disorganized or catatonic) that do not meet the criteria for any specific mental disorder.

Common examples of this category include:

- (1) Psychoses with unusual features, e.g., monosymptomatic delusion of bodily change without accompanying impairment in functioning; persistent auditory hallucinations as the only disturbance; transient psychotic episodes associated with the menstrual cycle.
- (2) "Postpartum psychoses" that do not meet the criteria for an Organic Mental Disorder, Schizophreniform Disorder, Paranoid Disorder, or Affective Disorder.
- (3) Psychoses that would be classified elsewhere except that the duration is less than two weeks, e.g., the symptomatology of a Schizophreniform Disorder, but lasting only three days and there is no precipitating psychosocial stressor.
- (4) Psychoses about which there is inadequate information to make a more specific diagnosis. (This is preferable to Diagnosis Deferred, and can be changed if more information becomes available.)
- (5) Psychoses with confusing clinical features that make a more specific diagnosis impossible.

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Affective Disorders*

The essential feature of this group of disorders is a disturbance of mood, accompanied by a full or partial manic or depressive syndrome, that is not due to any other physical or mental disorder. Mood refers to a prolonged emotion that colors the whole psychic life; it generally involves either depression or elation. The manic and depressive syndromes each consist of characteristic symptoms that tend to occur together.

In other classifications these disorders are grouped in various categories, including Affective, Personality, and Neurotic Disorders.

Subclassification of Affective Disorders. The classification of Affective Disorders in DSM-III differs from many other classifications based on such dichotomous distinctions as neurotic vs. psychotic or endogenous vs. reactive.

In this manual the class Affective Disorders is divided into Major Affective Disorders, in which there is a full affective syndrome; Other Specific Affective Disorders, in which there is only a partial affective syndrome of at least two years' duration; and finally, Atypical Affective Disorders, a category for those affective disorders that cannot be classified in either of the two specific subclasses.

Major Affective Disorders include Bipolar Disorder and Major Depression, which are distinguished by whether or not there has ever been a manic episode. A category of Manic Disorder is not included in this classification; instead, when there has been one or more manic episodes, with or without a history of a major depressive episode, the category Bipolar Disorder is used. Bipolar Disorder is subclassified at the fourth digit as Mixed, Manic, or Depressed; Major Depression is subclassified at the fourth digit as Single Episode or Recurrent. The current episode is further subclassified at the fifth digit to reflect certain characteristics such as the presence of psychotic features and, in the case of a major depressive episode, the presence of Melancholia.†

Other Specific Affective Disorders include Cyclothymic Disorder and Dysthymic Disorder. In Cyclothymic Disorder there are symptoms characteristic of both the depressive and the manic syndromes, but they are not of sufficient severity and duration to meet the criteria for major depressive or manic episodes. In Dysthymic Disorder the symptoms are not of sufficient severity and duration

* The proper descriptive term for this group of disorders should be "Mood Disorders"; however, common usage and historical continuity favor retention of the term "Affective Disorders."

† A term from the past, in this manual used to indicate a typically severe form of depression that is particularly responsive to somatic therapy. The clinical features that characterize this syndrome have been referred to as "endogenous." Since the term "endogenous" implies, to many, the absence of precipitating stress, a characteristic not always associated with this syndrome, the term "endogenous" is not used in DSM-III.

to meet the criteria for a major depressive episode, and there have been no hypomanic periods. A theoretically possible third disorder in this group is Chronic Hypomanic Disorder, which would require hypomanic features of at least two years' duration, but not depressive periods; the existence of such a syndrome has not been well enough established to warrant inclusion as a distinct disorder. These chronic disorders may have a superimposed episode of Major Affective Disorder, in which case both diagnoses should be recorded.

MAJOR AFFECTIVE DISORDERS*

The essential feature is an illness involving either a manic episode (see below) or a major depressive episode (p. 210). These major affective episodes are not diagnosed if the affective disturbance is due to an Organic Mental Disorder or if it is superimposed on Schizophrenia.

Manic Episode

The essential feature is a distinct period when the predominant mood is either elevated, expansive, or irritable and when there are associated symptoms of the manic syndrome. These symptoms include hyperactivity, pressure of speech, flight of ideas, inflated self-esteem, decreased need for sleep, distractibility, and excessive involvement in activities that have a high potential for painful consequences, which is not recognized.

The elevated mood may be described as euphoric, unusually good, cheerful, or high; often has an infectious quality for the uninvolved observer; but is recognized as excessive by those who know the individual well. The expansive quality of the mood disturbance is characterized by unceasing and unselective enthusiasm for interacting with people and seeking involvement with other aspects of the environment. Although elevated mood is considered the prototypical symptom, the predominant mood disturbance may be irritability, which may be most apparent when the individual is thwarted.

The hyperactivity often involves excessive planning of and participation in multiple activities (e.g., sexual, occupational, political, religious). Almost invariably there is increased sociability, which includes efforts to renew old acquaintanceships and calling friends at all hours of the night. The intrusive, domineering, and demanding nature of these interactions is not recognized by the individual. Frequently, expansiveness, unwarranted optimism, grandiosity, and lack of judgment lead to such activities as buying sprees, reckless driving, foolish business investments, and sexual behavior unusual for the individual. Often the activities have a disorganized, flamboyant, or bizarre quality, for example, dressing in colorful or strange garments, wearing excessive, poorly applied make-up, or distributing candy, money, or advice to passing strangers.

Manic speech is typically loud, rapid, and difficult to interrupt. Often it is

*The organization of the text for the Major Affective Disorders departs from the usual method of presentation in order to avoid redundancy. The essential features, associated features, differential diagnosis and diagnostic criteria of manic and major depressive episodes are described first. Age at onset, course and other features of both manic and major depressive episodes are discussed next. Finally, the diagnostic criteria for the specific Major Affective Disorders are listed.

full of jokes, puns, plays on words, and amusing irrelevancies. It may become theatrical, with dramatic mannerisms and singing. Sounds rather than meaningful conceptual relationships may govern word choice (clanging). If the mood is more irritable than expansive, there may be complaints, hostile comments, and angry tirades.

Frequently there is flight of ideas, i.e., a nearly continuous flow of accelerated speech with abrupt changes from topic to topic, usually based on understandable associations, distracting stimuli, or plays on words. When flight of ideas is severe, the speech may be disorganized and incoherent. However, loosening of associations and incoherence may occur even when there is no flight of ideas, particularly if the individual is on medication.

Distractibility is usually present and manifests itself as rapid changes in speech or activity as a result of responding to various irrelevant external stimuli, such as background noise or signs or pictures on the wall.

Characteristically, there is inflated self-esteem, ranging from uncritical self-confidence to marked grandiosity, which may be delusional. For instance, advice may be given on matters about which the individual has no special knowledge, such as how to run a mental hospital or the United Nations. Despite a lack of any particular talent, a novel may be started, music composed, or publicity sought for some impractical invention. Grandiose delusions involving a special relationship to God or some well-known figure from the political, religious, or entertainment world are common.

Almost invariably there is a decreased need for sleep; the individual awakens several hours before the usual time, full of energy. When the sleep disturbance is severe, the individual may go for days without any sleep at all and yet not feel tired.

The term "hypomania" is used to describe a clinical syndrome that is similar to, but not as severe as, that described by the term "mania" or "manic episode."

Associated features. A common associated feature is lability of mood, with rapid shifts to anger or depression. The depression, expressed by tearfulness, suicidal threats, or other depressive symptoms, may last moments, hours, or, more rarely, days. Occasionally the depressive and manic symptoms intermingle, occurring at the same time; or they may alternate rapidly within a few days. Less often, in Bipolar Disorder, Mixed, the depressive symptoms are more prominent and last at least a full day, and there is the full symptom picture of manic and major depressive episodes.

When delusions or hallucinations are present, their content is usually clearly consistent with the predominant mood (mood-congruent). God's voice may be heard explaining that the individual has a special mission. Persecutory delusions may be based on the idea that the individual is being persecuted because of some special relationship or attribute. Less commonly, the content of the hallucinations or delusions has no apparent relationship to the predominant mood (mood-incongruent). The usefulness of the distinction between mood-congruent and mood-incongruent psychotic features is controversial.

Differential diagnosis of manic episode. **Organic Affective Syndromes** with mania may be due to such substances as amphetamines or steroids, or to some other known organic factor, such as multiple sclerosis. The diagnosis of a manic episode should be made only if a known organic etiology can be excluded. For further discussion, see p. 117.

In **Schizophrenia, Paranoid Type**, there may be irritability and anger that are difficult to distinguish from similar features in a manic episode. In such instances it may be necessary to rely on features that, on a statistical basis, are associated differentially with the two conditions. For example, the diagnosis of a manic episode is more likely if there is a family history of Affective Disorder, good premorbid adjustment, and a previous episode of an Affective Disorder from which there was complete recovery.

The diagnosis **Schizoaffective Disorder** may be made whenever the clinician is unable to make a differential diagnosis between manic episode and Schizophrenia. Although no criteria for Schizoaffective Disorder are provided in this manual, several examples of clinical situations in which this diagnosis might be appropriate are given on p. 202.

In **Cyclothymic Disorder** there are hypomanic periods, but the full manic syndrome is not present. However, in some instances a manic episode may be superimposed on Cyclothymic Disorder. In such cases both Bipolar Disorder and Cyclothymic Disorder should be diagnosed, since it is likely that when the individual recovers from the manic episode, the Cyclothymic Disorder will persist.

Diagnostic criteria for a manic episode

A. One or more distinct periods with a predominantly elevated, expansive, or irritable mood. The elevated or irritable mood must be a prominent part of the illness and relatively persistent, although it may alternate or intermingle with depressive mood.

B. Duration of at least one week (or any duration if hospitalization is necessary), during which, for most of the time, at least three of the following symptoms have persisted (four if the mood is only irritable) and have been present to a significant degree:

- (1) increase in activity (either socially, at work, or sexually) or physical restlessness
- (2) more talkative than usual or pressure to keep talking
- (3) flight of ideas or subjective experience that thoughts are racing
- (4) inflated self-esteem (grandiosity, which may be delusional)
- (5) decreased need for sleep
- (6) distractibility, i.e., attention is too easily drawn to unimportant or irrelevant external stimuli
- (7) excessive involvement in activities that have a high potential for painful consequences which is not recognized, e.g., buying sprees, sexual indiscretions, foolish business investments, reckless driving

C. Neither of the following dominates the clinical picture when an affective syndrome is absent (i.e., symptoms in criteria A and B above):

- (1) preoccupation with a mood-incongruent delusion or hallucination (see definition below)
- (2) bizarre behavior

D. Not superimposed on either Schizophrenia, Schizophreniform Disorder, or a Paranoid Disorder.

E. Not due to any Organic Mental Disorder, such as Substance Intoxication.

(**Note:** A hypomanic episode is a pathological disturbance similar to, but not as severe as, a manic episode.)

Fifth-digit code numbers and criteria for subclassification of manic episode

6— In Remission. This fifth-digit category should be used when in the past the individual met the full criteria for a manic episode but now is essentially free of manic symptoms or has some signs of the disorder but does not meet the full criteria. The differentiation of this diagnosis from no mental disorder requires consideration of the period of time since the last episode, the number of previous episodes, and the need for continued evaluation or prophylactic treatment.

4— With Psychotic Features. This fifth-digit category should be used when there apparently is gross impairment in reality testing, as when there are delusions or hallucinations or grossly bizarre behavior. When possible, specify whether the psychotic features are mood-incongruent. (The non-ICD-9-CM fifth-digit 7 may be used instead to indicate that the psychotic features are mood-incongruent; otherwise, mood-congruence may be assumed.)

Mood-congruent Psychotic Features: Delusions or hallucinations whose content is entirely consistent with the themes of inflated worth, power, knowledge, identity, or special relationship to a deity or famous person; flight of ideas without apparent awareness by the individual that the speech is not understandable.

Mood-incongruent Psychotic Features: Either (a) or (b):

- (a) Delusions or hallucinations whose content does not involve themes of either inflated worth, power, knowledge, identity, or special relationship to a deity or famous person. Included are such symptoms as persecutory delusions, thought insertion, and delusions of being controlled, whose content has no apparent relationship to any of the themes noted above.

(b) Any of the following catatonic symptoms: stupor, mutism, negativism, posturing.

2- Without Psychotic Features. Meets the criteria for manic episode, but no psychotic features are present.

0- Unspecified.

Major Depressive Episode

The essential feature is either a dysphoric mood, usually depression, or loss of interest or pleasure in all or almost all usual activities and pastimes. This disturbance is prominent, relatively persistent, and associated with other symptoms of the depressive syndrome. These symptoms include appetite disturbance, change in weight, sleep disturbance, psychomotor agitation or retardation, decreased energy, feelings of worthlessness or guilt, difficulty concentrating or thinking, and thoughts of death or suicide or suicidal attempts.

An individual with a depressive syndrome will usually describe his or her mood as depressed, sad, hopeless, discouraged, down in the dumps, or in terms of some other colloquial variant. Sometimes, however, the mood disturbance may not be expressed as a synonym for depressive mood but rather as a complaint of "not caring anymore," or as a painful inability to experience pleasure. In a child with a depressive syndrome there may not be complaints of any dysphoric mood, but its existence may be inferred from a persistently sad facial expression.

Loss of interest or pleasure is probably always present in a major depressive episode to some degree, but the individual may not complain of this or even be aware of the loss, although family members may notice it. Withdrawal from friends and family and neglect of avocations that were previously a source of pleasure are common.

Appetite is frequently disturbed, usually with loss of appetite, but occasionally with increased appetite. When loss of appetite is severe, there may be significant weight loss or, in the case of children, failure to make expected weight gains. When appetite is markedly increased there may be significant weight gain.

Sleep is commonly disturbed, more frequently with insomnia present, but sometimes with hypersomnia. The insomnia may involve difficulty falling asleep (initial insomnia), waking up during sleep and then returning to sleep only with difficulty (middle insomnia), or early morning awakening (terminal insomnia).

Psychomotor agitation takes the form of inability to sit still, pacing, hand-wringing, pulling or rubbing of hair, skin, clothing, or other objects, outbursts of complaining or shouting, or pressure of speech. Psychomotor retardation may take the form of slowed speech, increased pauses before answering, low or monotonous speech, slowed body movements, a markedly decreased amount of speech (poverty of speech), or muteness. (In children there may be hypoactivity rather than psychomotor retardation.) A decrease in energy level is almost

invariably present, and is experienced as sustained fatigue even in the absence of physical exertion. The smallest task may seem difficult or impossible to accomplish.

The sense of worthlessness varies from feelings of inadequacy to completely unrealistic negative evaluations of one's worth. The individual may reproach himself or herself for minor failings that are exaggerated and search the environment for cues confirming the negative self-evaluation. Guilt may be expressed as an excessive reaction to either current or past failings or as exaggerated responsibility for some untoward or tragic event. The sense of worthlessness or guilt may be of delusional proportions.

Difficulty in concentrating, slowed thinking, and indecisiveness are frequent. The individual may complain of memory difficulty and appear easily distracted.

Thoughts of death or suicide are common. There may be fear of dying, the belief that the individual or others would be better off dead, wishes to die, or suicidal plans or attempts.

Associated features. Common associated features include depressed appearance, tearfulness, feelings of anxiety, irritability, fear, brooding, excessive concern with physical health, panic attacks, and phobias.

When delusions or hallucinations are present, their content is usually clearly consistent with the predominant mood (mood-congruent). A common delusion is that one is being persecuted because of sinfulness or some inadequacy. There may be nihilistic delusions of world or personal destruction, somatic delusions of cancer or other serious illness, or delusions of poverty. Hallucinations, when present, are usually transient and not elaborate, and may involve voices that berate the individual for his or her shortcomings or sins.

Less commonly the content of the hallucinations or delusions has no apparent relationship to the mood disturbance (mood-incongruent). This is particularly the case with persecutory delusions, in which the individual may be at a loss to explain why he or she should be the object of persecution. The usefulness of the distinction between mood-congruent and mood-incongruent psychotic features is controversial.

Age-specific associated features. Although the essential features of a major depressive episode are similar in infants, children, adolescents, and adults, there are differences in the associated features.

In prepubertal children separation anxiety may develop and cause the child to cling, to refuse to go to school, and to fear that he or she or the parents will die. A previous history of separation anxiety may result in more intense anxiety symptoms with the onset of a major depressive episode.

In adolescent boys negativistic or frankly antisocial behavior may appear. Feelings of wanting to leave home or of not being understood and approved of, restlessness, grouching, and aggression are common. Sulkiness, a reluctance to cooperate in family ventures, and withdrawal from social activities, with retreat to one's room, are frequent. School difficulties are likely. There may be

inattention to personal appearance and increased emotionality, with particular sensitivity to rejection in love relationships. Substance Abuse may develop.

In elderly adults there may be symptoms suggesting Dementia, such as disorientation, memory loss, and distractibility. Loss of interest or pleasure in the individual's usual activities may appear as apathy; difficulty in concentration as inattentiveness. These symptoms make the differential diagnosis of "pseudo-dementia" (due to depression) from true Dementia (an Organic Mental Disorder) particularly difficult (p. 111).

Differential diagnosis of major depressive episode. An **Organic Affective Syndrome with depression** may be due to substances such as reserpine, to infectious diseases such as influenza, or to hypothyroidism. Only by excluding organic etiology can one make the diagnosis of a major depressive episode. For further discussion, see p. 117.

Primary Degenerative Dementia or Multi-infarct Dementia, because of the presence of disorientation, apathy, and complaints of difficulty concentrating or of memory loss, may be difficult to distinguish from a major depressive episode occurring in the elderly. If the features suggesting a major depressive episode are at least as prominent as those suggesting Dementia, it is best to diagnose a major depressive episode and assume that the features suggesting Dementia represent a pseudo-dementia that is a manifestation of the major depressive episode. In such cases the successful treatment of the major depressive episode often results in the disappearance of the symptoms suggesting Dementia. If the features suggesting Dementia are more prominent than the depressive features, the diagnosis should be the appropriate form of Dementia, but the presence of depressive features should be noted.

If a **psychological reaction to the functional impairment associated with a physical illness** that does not involve the central nervous system causes a depression that meets the full criteria for a major depressive episode, the Major Depression should be recorded on Axis I, the physical disorder on Axis III, and the severity of the psychosocial stressor on Axis IV. Examples would include the psychological reaction to the amputation of a leg or to the development of a life-threatening or incapacitating illness.

In **Schizophrenia** there is usually considerable depressive symptomatology. If an episode of depression follows an episode of Schizophrenia and is superimposed upon the residual phase of Schizophrenia, the additional diagnosis of either Atypical Depression or Adjustment Disorder with Depressed Mood may be made, but not Major Depression. An individual with a major depressive episode may have psychotic symptoms; however, the diagnosis of Schizophrenia is made in the presence of a full depressive syndrome only if the affective symptoms follow the psychotic symptoms or are brief relative to the duration of the psychotic symptoms. An individual with Schizophrenia, Catatonic Type, may appear to be withdrawn and depressed, and it may be difficult to distinguish this condition from Major Depression with psychomotor retardation. In such instances it may be necessary to rely on features that on a statistical basis are associated differentially with the two disorders. For example, the diagnosis of a major depressive episode is more likely if there is a family history

of Affective Disorder, good premorbid adjustment, and a previous episode of affective disturbance from which there was complete recovery.

The diagnosis of **Schizoaffective Disorder** can be made whenever the clinician is unable to make a differential diagnosis between a major depressive episode and Schizophrenia. Although no criteria for Schizoaffective Disorder are provided in this manual, several examples of clinical situations in which this diagnosis might be appropriate are given on p. 202.

In **Dysthymic** and **Cyclothymic Disorders** there are features of the depressive syndrome, but they are not of sufficient severity and duration to meet the criteria for a major depressive episode. However, in some instances, a major depressive episode is superimposed on one of these disorders. In such cases both diagnoses should be recorded, since it is likely that after recovering from the major depressive episode, either a Dysthymic or a Cyclothymic Disorder will persist.

Chronic mental disorders, such as **Obsessive Compulsive Disorder** or **Alcohol Dependence**, when associated with depressive symptoms, may suggest a Major Depression. The additional diagnosis of Major Depression should be made only if the full depressive syndrome is present and persistent. In such instances both the chronic mental disorder and the superimposed Major Depression should be recorded.

In **Separation Anxiety Disorder**, depressive symptoms are common, but if the full depressive syndrome is not present, only Separation Anxiety Disorder should be diagnosed. On the other hand, children with Separation Anxiety Disorder may develop a superimposed major depressive episode, in which case both diagnoses should be made.

Uncomplicated Bereavement is distinguished from a major depressive episode and is not considered a mental disorder even when associated with the full depressive syndrome (see p. 333). However, if bereavement is unduly severe or prolonged, the diagnosis may be changed to Major Depression.

Diagnostic criteria for major depressive episode

A. Dysphoric mood or loss of interest or pleasure in all or almost all usual activities and pastimes. The dysphoric mood is characterized by symptoms such as the following: depressed, sad, blue, hopeless, low, down in the dumps, irritable. The mood disturbance must be prominent and relatively persistent, but not necessarily the most dominant symptom, and does not include momentary shifts from one dysphoric mood to another dysphoric mood, e.g., anxiety to depression to anger, such as are seen in states of acute psychotic turmoil. (For children under six, dysphoric mood may have to be inferred from a persistently sad facial expression.)

B. At least four of the following symptoms have each been present nearly every day for a period of at least two weeks (in children under six, at least three of the first four).

- (1) poor appetite or significant weight loss (when not dieting) or increased appetite or significant weight gain (in children under six, consider failure to make expected weight gains)
- (2) insomnia or hypersomnia
- (3) psychomotor agitation or retardation (but not merely subjective feelings of restlessness or being slowed down) (in children under six, hypoactivity)
- (4) loss of interest or pleasure in usual activities, or decrease in sexual drive not limited to a period when delusional or hallucinating (in children under six, signs of apathy)
- (5) loss of energy; fatigue
- (6) feelings of worthlessness, self-reproach, or excessive or inappropriate guilt (either may be delusional)
- (7) complaints or evidence of diminished ability to think or concentrate, such as slowed thinking, or indecisiveness not associated with marked loosening of associations or incoherence
- (8) recurrent thoughts of death, suicidal ideation, wishes to be dead, or suicide attempt

C. Neither of the following dominate the clinical picture when an affective syndrome is absent (i.e., symptoms in criteria A and B above):

- (1) preoccupation with a mood-incongruent delusion or hallucination (see definition below)
- (2) bizarre behavior

D. Not superimposed on either Schizophrenia, Schizophreniform Disorder, or a Paranoid Disorder.

E. Not due to any Organic Mental Disorder or Uncomplicated Bereavement.

Fifth-digit code numbers and criteria for subclassification of major depressive episode

(When psychotic features and Melancholia are present the coding system requires that the clinician record the single most clinically significant characteristic.)

6— In Remission. This fifth-digit category should be used when in the past the individual met the full criteria for a major depressive episode but now is essentially free of depressive symptoms or has some signs of the disorder but does not meet the full criteria.

4— With Psychotic Features. This fifth-digit category should be used when there apparently is gross impairment in reality testing, as when there are delusions or hallucinations, or depressive stupor (the individual

is mute and unresponsive). When possible, specify whether the psychotic features are mood-congruent or mood-incongruent. (The non-ICD-9-CM fifth-digit 7 may be used instead to indicate that the psychotic features are mood-incongruent; otherwise, mood-congruence may be assumed.)

Mood-congruent Psychotic Features. Delusions or hallucinations whose content is entirely consistent with the themes of either personal inadequacy, guilt, disease, death, nihilism, or deserved punishment; depressive stupor (the individual is mute and unresponsive).

Mood-incongruent Psychotic Features. Delusions or hallucinations whose content does not involve themes of either personal inadequacy, guilt, disease, death, nihilism, or deserved punishment. Included here are such symptoms as persecutory delusions, thought insertion, thought broadcasting, and delusions of control, whose content has no apparent relationship to any of the themes noted above.

3- With Melancholia. Loss of pleasure in all or almost all activities, lack of reactivity to usually pleasurable stimuli (doesn't feel much better, even temporarily, when something good happens), and at least three of the following:

- (a) distinct quality of depressed mood, i.e., the depressed mood is perceived as distinctly different from the kind of feeling experienced following the death of a loved one
- (b) the depression is regularly worse in the morning
- (c) early morning awakening (at least two hours before usual time of awakening)
- (d) marked psychomotor retardation or agitation
- (e) significant anorexia or weight loss
- (f) excessive or inappropriate guilt

2- Without Melancholia

0- Unspecified

OTHER FEATURES OF BOTH MANIC AND MAJOR DEPRESSIVE EPISODES

Age at onset. The first manic episode of Bipolar Disorder typically occurs before age 30. Major Depression may begin at any age, including infancy, and the age at onset is fairly evenly distributed throughout adult life.

Course. Manic episodes typically begin suddenly, with a rapid escalation of symptoms over a few days. The episodes usually last from a few days to months and are briefer and end more abruptly than major depressive episodes.

Most individuals who have a disorder characterized by one or more manic episodes (Bipolar Disorder) will eventually have a major depressive episode.

The onset of a major depressive episode is variable, the symptoms usually developing over a period of days to weeks; but in some cases it may be sudden (e.g., when associated with a severe psychosocial stress). In some instances prodromal symptoms—e.g., generalized anxiety, panic attacks, phobias, or mild depressive symptoms—may occur over a period of several months. It is estimated that over 50% of individuals with a Major Depression, Single Episode, will eventually have another major depressive episode, thus meeting the criteria for Major Depression, Recurrent. Individuals with Major Depression, Recurrent, are at greater risk of developing Bipolar Disorder than are those with a single episode of Major Depression.

In Bipolar Disorder the initial episode is often manic. Both the manic and the major depressive episodes are more frequent and shorter than the major depressive episodes in Major Depression. Frequently a manic or major depressive episode is immediately followed by a short episode of the other kind. In rare cases, over long periods of time there is an alternation of the two kinds of episodes without an intervening period of normal mood (cycling).

The course of Major Affective Disorders is variable. Some individuals have episodes separated by many years of normal functioning; others have clusters of episodes; and still others have an increased frequency of episodes as they grow older. Usually functioning returns to the premorbid level between episodes. However, in 20% to 35% of cases there is a chronic course with considerable residual symptomatic and social impairment. This is more likely when there are frequent recurrent episodes.

Impairment. In manic episodes there are usually considerable impairment in both social and occupational functioning and a need for protection from the consequences of poor judgment or hyperactivity.

In major depressive episodes the degree of impairment varies, but there is always some interference in social and occupational functioning. If impairment is severe, the individual may be totally unable to function socially or occupationally, or even to feed or clothe himself or herself or maintain minimal personal hygiene.

Complications. The most common complications of a manic episode are Substance Abuse and the consequences of actions resulting from impaired judgment, such as financial losses and illegal activities.

The most serious complication of a major depressive episode is suicide.

Predisposing factors. Chronic physical illness, Alcohol Dependence, Cyclothymic and Dysthymic Disorders apparently predispose to the development of a Major Affective Disorder.

Frequently an episode of Major Affective Disorder follows a psychosocial stressor. If an individual has recurrent episodes, however, subsequent episodes may occur apparently without precipitating factors.